



“The inspiration (for UChek) was the mobile phone and how it has revolutionized small businesses in India. Healthcare can also be revolutionary in the same way, and provide more affordable and accessible services to common people, if we are smart with technology.”

accuracy and reliability levels. The uChek units also provide advanced data analytics using the cloud.

How does it work?

For uChek, right from the start, technology enables everything: The test is done by imaging urine and blood reagent strips on Android powered devices. The data is processed on the mobile device, providing a real time reading and recommendation on the fly, to the health worker doing the testing in front of the patient. The doctor is consulted over the phone/ on video. All the data and the test report, as well as the doctor's recommendation is logged on the mobile device, and simultaneously, syncs with the cloud, whenever network connectivity is available. If network is not available, the system will carry on regardless and wait to upload data whenever network is available. All the data can be accessed by program managers and different levels of the health system, at all locations. Everything is geotagged with GPS and timestamped, so real time decisions are made backed by data that is current and relevant. All this runs on the internet infrastructure, most of which is open and freely available.

Myshkin is extremely bullish about UChek and other affordable healthcare ventures he's working on currently. When we asked him about how this will benefit India, Myshkin said, “The uChek and similar mobile based healthcare solutions we are developing cost 1/10th of what other more “traditional” diagnostic equipment cost, and thus make healthcare available for a new strata of society, that had otherwise been underserved or not served at all. Going forward, we think that health is going to follow telecom: In India, instead of going the way of expen-

sive landline infrastructure (telephone poles, lines, etc as in other developed nations) whole districts and states started accessing voice and data on mobile, over the air. Mobile has leapfrogged ahead at an unprecedented rate in India. I believe in Dr Mashelkar's words “Affordable Excellence” and that India will be the first country where per capita income and quality of healthcare will be totally separate: i.e everyone will have access to affordable, high quality healthcare, the same way they have access to affordable mobile talktime.”

Part of the new India, which says no to cushy well-paid jobs at MNCs or government desks and cherishes entrepreneurship and starting your own venture, Myshkin suggests that India has never lacked in entrepreneurial spirit. He points out that the “Old India was one of the power centres of global trade and commerce because of the symbiosis between trade, religion and science. We

seem to have lost our way somewhat in the last few hundred years, but it seems we are making a come-back now! I think an open society does not banish the failures, it cherishes them. Each failure is a better teacher than any professor or guru in any university, and open and innovation driven societies have learnt how to harness the power of failure to power future successes, in a systematic way. I don't think we are there yet, but the difference in attitudes are already perceptible: From IAS and MBA, the new thinking now is slowly changing to *kuch hatke karna hai, kuch jabardast karke dikhana hai.*”

Going beyond just entrepreneurship and harnessing technology to its fullest, saving costs on already available medical diagnostic tools, Myshkin feels there's a strong need for indigenously developed solutions to tackle India's medicare and healthcare sectors. Until we do that, there can be no true salvation for Indian healthcare and the problems that plague the medicare system (or its lack thereof) in our country.

He continues to mention that most of India's medicare “systems” have been borrowed from the West. “Sometimes this works, but other times, India needs its own thinking. For example, all equipment in medicine has been invented in Europe or US, mostly. But this solves the problems of the West, and has been designed with their existing infrastructure in mind. India is different and Indian health care problems need special thinking,” says Myshkin. He continues to urge all engineers, designers and doctors to “think different, and think out of the box to solve India's health problems.” How many of you reading will take up the cause of doing something truly path-breaking to disrupt medicare in the country?

WHAT'S THE DIFFERENCE BETWEEN THE INDIA YOU WERE BORN INTO AND THE INDIA YOU ARE LIVING IN RIGHT NOW?

“The difference is that the old India was always looking for a free lunch. We always looked to other countries to help us, and we always looked to even our own government to “save us”, whereas now, the young India is looking inwards and starting to come up with indigenous solutions, innovative ways to address age old problems. Truly transformative change is only possible if it comes from within, is driven organically. And that is what is happening in India, as a whole new generation of doers is getting to work, getting things done in their own innovative way.”